Comment Letter

CMS-0042 – NC – Request for Information: Health Technology Ecosystem

Submitted by: Chicago Pacific Founders
Re: Request for Information: Health Technology Ecosystem (CMS-0042-NC)
Date: 16 June 2025

To: **Stephanie Carlton**, Deputy Administrator of the Centers for Medicare & Medicaid Services and **Steven Posnack**, Acting Assistant Secretary for Technology Policy, Acting National Coordinator for Health Information Technology.

Thank you for the opportunity to provide comments on the **Request for Information: Health Technology Ecosystem**. Chicago Pacific Founders (CPF) is a health sector focused mid-size private equity firm with a mission to accelerate health innovation and a focus in value-based care, data, technology and AI (see CPF Overview included as an attachment). Our comments focus on seven (7) sections of the RFI including seamless interoperability (selected questions from B. Patients and Caregivers, C. Providers, and F. Value-Based Care Organizations), patient-centric digital solutions (selected questions from B. Patients and Caregivers and D. Providers), digital identify management (selected questions from B. Patients and Caregivers, C. Providers, and E. Technology Vendors, Data Providers, and Networks), reducing provider burden (selected questions from B. Patients and Caregivers, C. Providers, E. Technology Vendors, Data Providers, and Networks, and F. Value-Based Care Organizations), supporting value-based care goals (selected questions from B. Patients and Caregivers and F. Value-Based Care Organizations), real-world evidence and data generation (selected questions from B. Patients and Caregivers, E. Technology Vendors, Data Providers, and Networks and F. Value-Based Care Organizations), and the technology ecosystem (selected questions from selected questions from B. Patients and Caregivers, C. Providers, E. Technology Vendors, Data Providers, and Networks and F. Value-Based Care Organizations)

We outline a cohesive pathway to a patient-centric, interoperable marketplace by (i) strengthening patient and caregiver access through a single SMART-on-FHIR credential, universal consent, and accessibility rules; (ii) streamlining provider operations with time-bound prior-authorization APIs, bulk-FHIR quality feeds, and digital-identity safeguards; (iii) aligning payer incentives via Blue Button expansion, shared-risk corridors, and Star-Ratings credits; (iv) catalyzing technology vendors, data providers, and networks through TEFCA participation, open APIs, evidence-lab pilots, and end-to-end cybersecurity; and (v) accelerating value-based care adoption with standardized multi-source data packages, TEFCA onboarding support, and reimbursement uplifts tied to real-time performance reporting.

Collectively, the recommendations give CMS practical levers—standards, incentives, and governance—to realize a seamlessly connected digital ecosystem and we describe an open innovation hub construct for CMS to consider in the executive summary that enables many of these recommendations. We are pleased to share our perspectives from our experience at the center of the health technology ecosystem working with health systems, payers, and value-based care organizations across the country.

Sincerely,

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Executive Summary

The Center for Medicare and Medicaid Services (CMS) should consider an **open innovation hub** for the **safe and responsible development of artificial intelligence (AI)** (Generative and Agentic) solutions that address Medicare and Medicaid **patient needs and alleviate provider administrative burden**.

The healthcare sector represents **20% of GDP** and is **woefully inefficient** with 40% of spend categorized as waste, abuse or fraud. **Hospital systems are on the brink of economic collapse** due to such things as rising labor costs, reimbursement pressure, and mix deterioration with no government bailout safety net, so there is a real urgency to the problem. **Access to care and health equity** are particularly acute for certain populations especially **MA and Medicaid**. Over the past five (5) years, total provider revenue tied to risk contracts has grown from $50B to $200B and this **value-based care (VBC) trend is expected to continue growing in a few dimensions beyond primary care** such as specialty focused models, exporting VBC to other areas through payer partnerships (e.g. hospital VBC), and expansion into other lines of business (LoBs) - traditional Medicare, Medicaid, and Commercial.

Chicago Pacific Founders (CPF) is a mid-sized, healthcare sector focused private equity firm with one of the **largest investments in VBC assets in the country** with over 1.6M full risk lives. **Our mission is to accelerate health innovation** which includes the following goals that are aligned with those of CMS - improved **patient engagement** and **access to care,** lower total **cost of care**, improved **quality of patient care outcomes,** and improved **provider satisfaction.**

AI is a foundational element of health innovation especially at a time when we will see an unprecedented change in how business processes are executed in healthcare with the application of generative and agentic AI. The purpose of the AI innovation hub is to identify the **highest value and hardest problems to solve** with an innovative suite of AI solutions and **CPF would be delighted to partner with CMS to use our portfolio companies to seed this innovation hub and serve as a "petri dish" for innovating around new solutions that are developed in a safe, responsible and trustworthy manner.**

The AI innovation hub has five (5) components:

1. One of the biggest challenges for healthcare AI innovation is **access to good datasets**. CPF could seed this data hub to develop a platform for innovation with **anonymized clinical and claims data on over 1.6M patients** across our VBC portfolio, **clinical data from 85M patients from OMNY** (a clinical data portfolio company), and supplement this with data from other data sources such as CMS, census, and social determinants of health (SDoH).

2. Leverage our healthcare data and technology expertise (**Impact Advisors and Allymar)** to develop functional and longitudinal data models by domain (i.e. patient engagement, clinical, claims etc.). **Impact Advisors** is a **healthcare management and IT consulting firm consistently ranked best in KLAS** over the last eighteen years and **Allymar** is a **technology enabled services platform** and contains extensive **provider and health plan domain expertise** – clinical, hospital operations, health plan, and financial management targeted toward applied AI to drive operational value.

3. We have a HITRUST certified **open technology orchestration platform** through a partner company **XSELL** to develop an **extensible and reusable** AI foundation built for **scale, high accuracy, and low latency.** This foundation contains both an **agentic AI platform** for patient and provider engagement using a **real-time eXpert Language Model (XLM)** built specifically for healthcare and contains the ability for **automated and accurate data labeling** for fine tuning models at scale.

4. Leverage the **extensive healthcare experience of Impact Advisors and Allymar** and "The AI Safe Standard of Care™" framework and playbook to develop AI products in a **safe, responsible, and trustworthy** manner to address **highest value problems** such as patient engagement, throughput, early disease detection, quality and HEDIS support, claims adjudication, and fraud / payment integrity.

5. The **AI open innovation hub** is focused on encouraging health systems, VBC providers, health technology startups, and academia to participate to develop, test, and prove the value of innovative AI solutions.

CPF would be happy to partner with CMS to host such an innovation hub with its combination of data, technology, consulting and VBC assets and partner with CMS' preferred ecosystem participants such as data providers, AI solutions providers, and health systems etc., to target healthcare's highest impact problems.

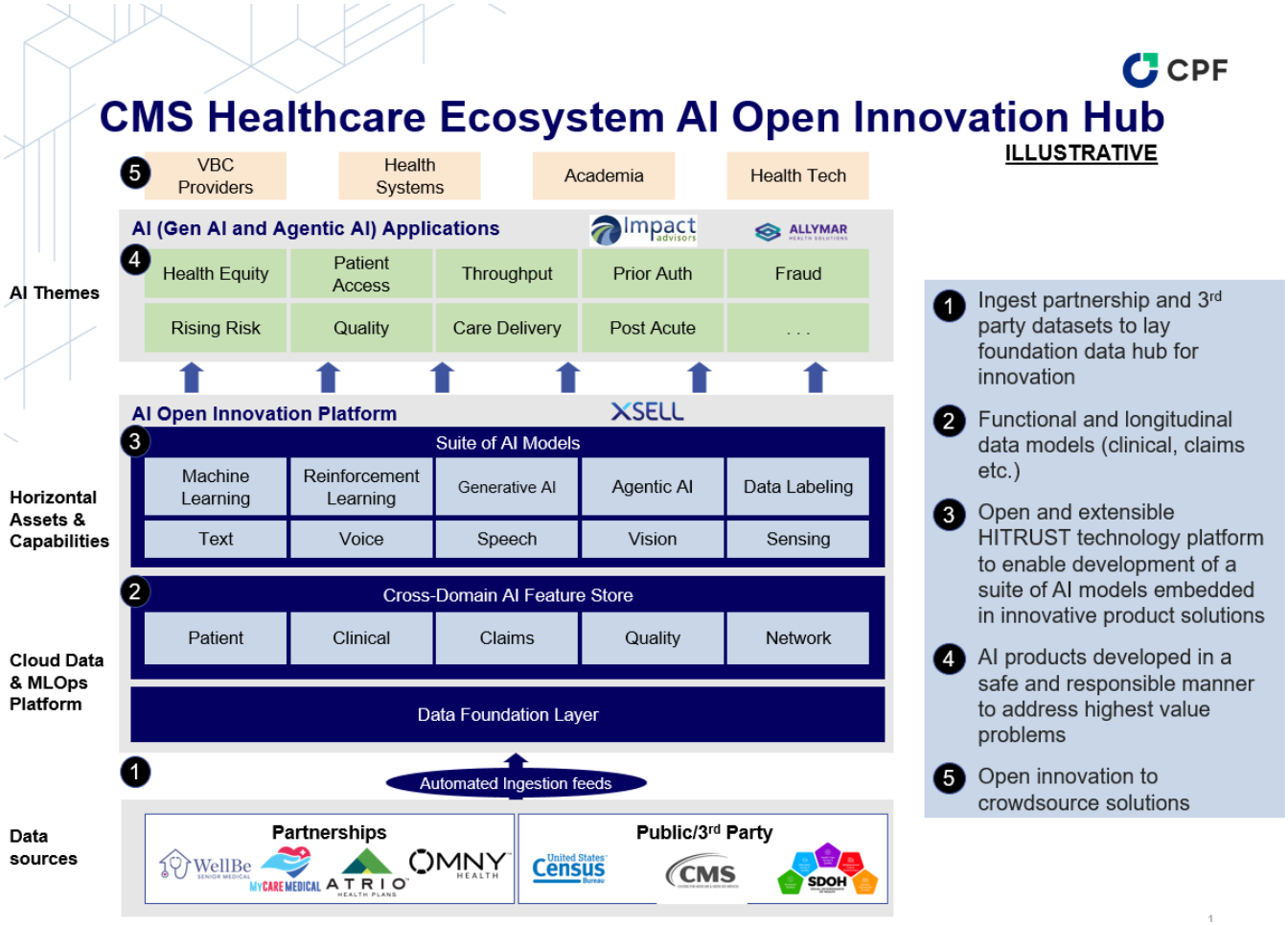

Figure 1: AI Open Innovation Hub

1. Seamless Interoperability

Fragmented, delayed, and non-standard data throttle digital innovation and drive costly manual work, even though Fast Healthcare Interoperability Resources (FHIR) application programming interfaces (APIs) and Trusted Exchange Framework and Common Agreement (TEFCA) connectivity are now mandated. Seamless interoperability would be an avenue for CMS to test and prove new innovations prior to scaling nationwide where we would pool de-identified clinical and claims data in a secure, certified platform, expose that data through verified APIs, and let developers build once and deploy nationwide—giving

patients one digital front door and providers modern automation. Impact Advisors has substantial experience implementing electronic health record (EHR) platforms and enabling interoperability among myriad digital health tools such as Epic, Cerner and other core platforms including, but not limited to, Workday, UKG, and Peoplesoft.

Key Interoperability Challenges

- **Fragmented Encounter Visibility:** Without a single, consent-aware feed of clinical and claims events, providers and payers miss real-time admissions, procedures, and lab results performed elsewhere that may undermine care coordination.
- **Manual Payer-to-Payer Onboarding:** New-member data still moves through labor-intensive chart chase instead of standards-based APIs, leaving incoming plans without problem lists, medication histories, or hierarchical condition categories for months, inflating risk-adjustment errors, and delaying care-management interventions.
- **Manual Prior Authorization:** Clinicians still phone or fax requests and wait days for a decision which may delay therapy starts and push patients to higher-acuity care settings without expedient resolution.
- **Data Integration:** The most significant challenge is integrating commercial MA payer files, EHR, health information exchange (HIE), labs, and pharmacy data. There are over 11 different payer file types (e.g. MAO-004, MMR, MOR) that need to be processed across multiple contracts and health plans in different states. Integration of EHR data (one or more), HIE data (usually several different ones by state) and operational data (e.g. CRM, call center, scheduling, etc.) with the claims data at a patient level is challenging.
- **Inconsistent Standards:** Non-government claims data feeds, especially MA, are significantly more complex than accountable care organizations (ACOs) (and formerly MSSPs). ACO data feeds come directly from CMS and are delivered in a steady and consistent format. Commercial health plans often send low quality data, including inconsistent file names, inaccurate membership counts, incorrect or old files, changing data definitions, and add new columns and files that will pass basic audit checks and can only be flagged for errors manually or with highly sophisticated automated rules logic.

Solution in Practice: MyCare Integrated Unified Member Database

MyCare, an MA full-risk VBC provider with over 35K patients, engineered a **Unified Member Database (UMD)** that stitches together commercial Medicare Advantage claims, EHR, HIE, laboratory, and pharmacy feeds to create a longitudinal, 360-degree profile of every member. Real-time identity resolution, data normalization, and rigorous data-quality approaches reconcile millions of data points into a single source of truth. These enriched datasets power a portfolio of AI-driven “next-best-action” engines that surface risk-stratified, clinically prioritized interventions to help care teams close care gaps, prevent avoidable admissions, and exceed value-based performance targets.

- **Integrated Patient Access:** A single, normalized data stream spanning CMS and all contracted payers’ feeds and provider-facing tools in near real time, eliminating duplicative diagnostics and accelerating clinical decision-making.
- **Optimized Provider Operations:** Automated integration of disparate data and reporting supports clinicians managing at-risk panels with an AI-native next-best-action guidance while also removing low-value clerical tasks with significant administrative burden that reallocates provider capacity to clinical care. These functions integrate natively with EHRs through APIs, preserving established workflows.

- **Responsive Population Management:** Continuously refreshed, analysis-ready data keeps risk scores, attribution, and quality metrics current, enabling timely interventions and more precise value-based contracts. Governed data pipelines ensure auditability for regulators and partners.
- **Comprehensive Cost Transparency:** A patient-level ledger reconciles every claim, rebate, and high-cost episode, establishing an accurate total-cost-of-care view for finance, actuarial, and benefit-design teams. Granular insight into cost drivers strengthens contract negotiations and compliance reporting.

Recommendations for CMS

- **(PR-7)** CMS can mandate that all commercial insurers adopt the same reporting standards and formats required by CMS for e.g., ACOs to ensure that administrative claims data are high quality and easy to integrate into provider workflows.
- **(PC-1, PC-3)** Mandate SMART 2.0 patient-facing OAuth endpoints tied to a single verified digital credential usable across payers, providers, and third-party apps
- **(PR-1, VB-1)** Require payers to accept and respond to FHIR prior-authorization requests within seventy-two hours as a condition of participation in Medicare Advantage and Medicaid managed-care programs.
- **(VB-2, VB-3)** Grant bonus credit in Medicare Advantage Star Ratings for monthly submission of bulk-FHIR quality feeds through TEFCA to CMS.
- **(VB-5, PR-3)** Authorize Medicare Advantage plans to pool high-cost curative-therapy risk through a CMS-administered reinsurance corridor. Require outcome-based annuity payments spread over five years, triggered by real-world evidence captured via TEFCA event feeds.
- **(PR-2, VB-4)** Establish a three-year TEFCA regulatory sandbox that waives information-blocking penalties for pilots in real-time bundled-payment adjudication using FHIR Payment Bundle resources.
- **(PC-4)** Require Innovation Hub participants to use IAL2 digital identity standards and operate under an independent ethics and privacy board to ensure trust and auditability.
- **(PC-5)** Create a patient therapy ledger that travels with the enrollee across plan switches and automatically reconciles payment obligations between the originating and receiving plans.

2. Patient-Centric Digital Solutions

Challenges

Delivering patient-centric digital solutions in healthcare is hindered by several persistent challenges that disproportionately affect Medicare beneficiaries, caregivers, and providers:

- **Digital Literacy and Accessibility:** Many seniors and rural Medicare beneficiaries face barriers due to limited digital skills, connectivity challenges, and complex health apps, reducing their ability to effectively engage with digital care tools.
- **Fragmented Data Access:** Disconnected healthcare systems prevent unified patient data views, complicating care coordination and patient engagement, particularly for those managing multiple providers or chronic conditions.

- **Poor Usability and Design:** Digital health tools frequently emphasize clinical or administrative needs, resulting in unintuitive interfaces that are difficult for seniors and other less tech-savvy users to navigate effectively.
- **Caregiver Overload:** Caregivers often lack integrated digital tools to track medications, manage appointments, and monitor health trends, adding complexity to their responsibilities.
- **Provider Administrative Burden:** Non-integrated digital tools require providers to handle manual data entry and navigate multiple systems, increasing administrative tasks and reducing time available for direct patient care.

These challenges underscore the need for digital solutions that are intuitive, equitable, and seamlessly integrated into both patient and provider workflows.

Solutions In Practice

XSELL (a CPF partner company) has developed and is implementing innovative, patient-centric digital solutions (with Atrio, a CPF MA Health Plan) that address these challenges, delivering measurable improvements in access, engagement, and usability. See below in Figure 2 for an example of how an agentic voice solution would optimize the overall Medicare Advantage member experience:

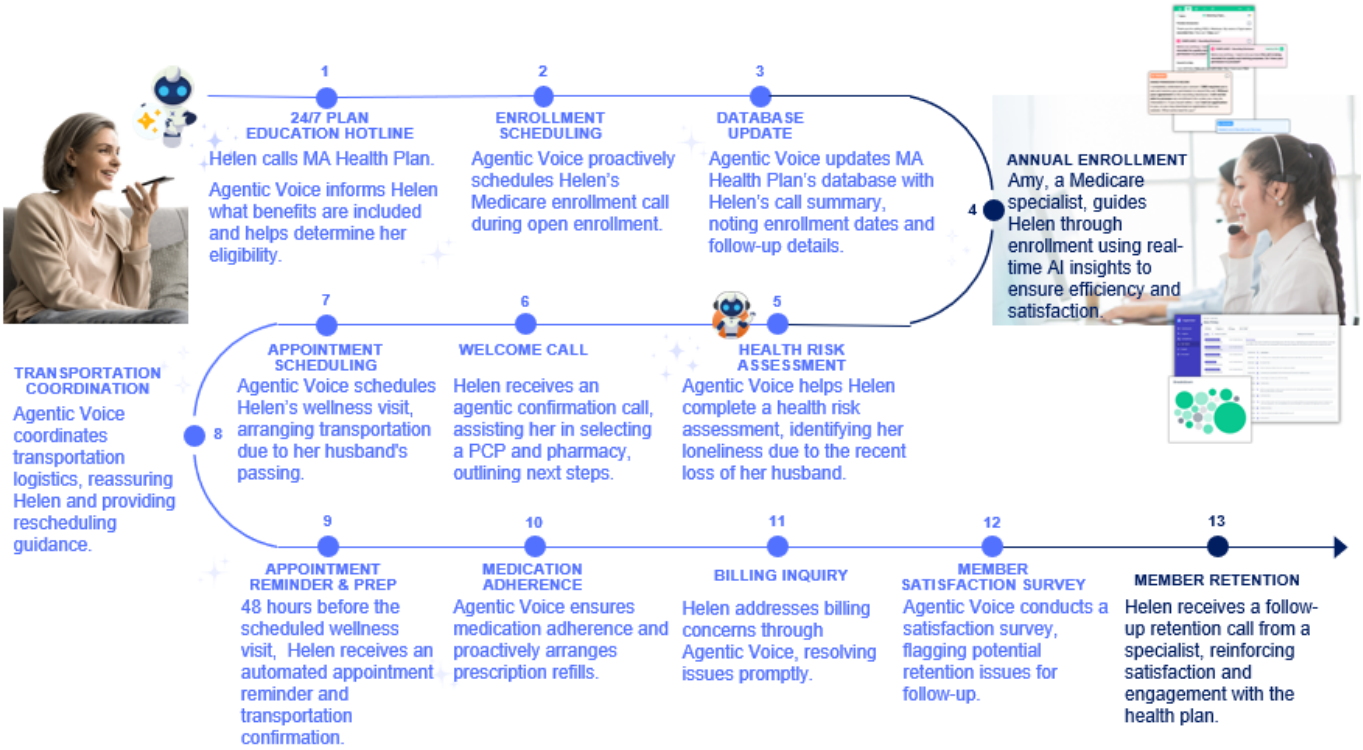

Figure 2: Agenic Voice Solution

Here we can see agentic voice technology reshapes member outreach with tailored guidance from providers, live data feeds, and enhanced care coordination. It unifies clinical updates, arranges logistics, supports medication adherence, and tracks satisfaction in real time. Resulting in higher engagement, fewer access barriers, and better outcomes for patients, caregivers, and providers.

Recommendations for CMS

To further advance patient-centric digital solutions, CMS should consider the following actions, directly addressing the RFI's focus areas:

- **(PC-6)** Establish guidelines requiring digital health tools to include simplified navigation, and accessibility features, such as voice commands, screen readers, and multilingual support. as a condition for CMS funding or certification.
- **(PR-1a)** Offer grants or reimbursement models for health systems that deploy caregiver-focused digital solutions, such as consolidated health management apps or caregiver-specific portals.
- **(PC-5a, PR-1)** Require certified health IT systems to support patient-initiated data exchange (e.g., via digital health wallets or APIs) and integrate patient-reported outcomes into provider workflows without manual entry.
- **(PC-6, PR-1)** Update telehealth reimbursement policies to promote equitable accessibility and ease of use for all healthcare consumers, ensuring equitable access for all Medicare beneficiaries.
- **(PC-5a)** Develop a single-consent (applicable across all provider processes and jurisdictions), standardized, user-friendly framework for digital health tools, leveraging FHIR and API-driven solutions to give patients control over their data with minimal complexity.

3. Digital Identity Management

Challenges

- **Fragmented Digital Identity and Access:** Clinicians must authenticate across multiple EHR, HIE, and payer portals, creating credential fatigue, surging help-desk demand, and a larger attack surface. Healthcare data breaches (affecting 500 or more individuals) are at an all-time high with over 700 in 2024 and each successful breach cost providers $9.8M on average, nearly double the cross-industry average of $4.9M – signaling the sensitive nature of health data, regulatory penalties and remediation complexities.
- **Manual Privacy and Consent Workflows:** Paper-based or ad-hoc e-signature processes slow referrals and leave patients—especially those with limited digital literacy—without real-time control over data sharing. Unmanaged copies of PHI also extend incident recovery where ransomware downtime now averages 19 days but only 22% of organizations restore operations in under seven days, adding significant costs, beyond the initial breach, for providers due to operational disruption.
- **Inconsistent Data-Sharing Policies and Governance:** Variable state regulations and payer requirements fragment longitudinal records, impede care coordination, and heighten information-blocking exposure. The sector has recorded the highest breach cost for 14 consecutive years, reflecting both regulatory complexity and persistent adversary focus.
- **Escalating Threats:** Ransomware exploiting vendor connections or unpatched software forces 37 % of providers to remain offline for more than a month during a ransomware attack meaning every third-party integration represents a material seven-figure risk that must be actively addressed.

Solutions In Practice

Impact Advisors has deployed innovative solutions that address these challenges.

- **Identity and Access Management / Trust Frameworks:** Implemented federated digital identity and single sign-on solutions at leading health systems. Leveraging NIST-compliant identity, continuous biometric authentication, and adaptive access controls, we reduced login-related support calls and improved clinician satisfaction by streamlining workflows, while multilingual interfaces and robust training ensured accessibility.
- **Privacy, Consent, and PHI Control:** FHIR-based consent management platforms and modular API-driven solutions empower patients to manage their data securely, increasing engagement and reducing administrative burdens. App attestation registries and third-party audits further ensure privacy and compliance for HIPAA and non-HIPAA actors.
- **Information Risk and Blocking Behavior:** Embedded mobile-optimized, multilingual complaint tools and secured audit trails, reducing blocking incidents through automated policy detection and real-time monitoring.
- **Cybersecurity and Resilience:** Full-lifecycle API security assessments, endpoint hardening, and continuous monitoring, in collaboration with leading partners, have established real-time threat detection, enhancing compliance with industry standards.

Recommendations for CMS

To build on these successes and advance the healthcare technology ecosystem, CMS should take the following actions:

- **(PC-14, PR-9, TD-3, TD-5)** Expand federated digital identity (e.g., Universal Patient ID) and single sign-on solutions, leveraging biometric authentication and adaptive controls to reduce credential fatigue and improve access for patients and providers.
- **(PC-6, TD-8, TD-10, PR-5)** Support the deployment of FHIR-based consent platforms and API-driven solutions, ensuring patients control their PHI while reducing administrative workloads, with a focus on accessibility features for diverse populations.
- **(PC-13, PA-7, PR-12, TD-14, TD-18)** Encourage the adoption of mobile-optimized complaint tools and audit systems to enhance transparency and compliance, addressing policy conflicts and improving data sharing.
- **(TD-15)** Invest in full-lifecycle security assessments and continuous monitoring to protect against threats and strengthen system resilience, collaborating with ecosystem partners to address supply chain vulnerabilities.
- **(TD-3, TD-5)** Incentivize technology solution providers to develop standardized APIs and frameworks to streamline data exchange, drawing from successful models like meaningful use to drive adoption.
- **(PR-10, PR-11, PA-3)** Encourage industry-wide efforts, including with payers and regulators, to align on standards and incentives, ensuring scalable and equitable technology deployment.
- **(PR-5, TD-13)** Provide incentives for vendor-led training programs to equip healthcare organizations with the skills needed to adopt and maintain advanced technologies securely.

4. Reducing Provider Burden

Challenges in Reducing Provider Burden

Providers across the healthcare landscape are grappling with an escalating burden that diverts attention from patient care.

- **Complexities of fragmented data access:** Providers struggle to navigate records due to complex systems of record creating significant workflow disruptions. Manual care coordination across care settings remains labor-intensive, while crafting and updating personalized care plans demands extensive manual effort.
- **Administrative overload:** Scheduling, notifications, and patient education further strain resources, compounded by the challenge of monitoring at-risk patients and addressing social determinants of health (SDoH) like housing, food insecurity or transportation.
- **Technology adoption:** One of the largest challenges is adoption lag, not technology. Providers are inundated with technology point solutions which fail to integrate as a single source of truth, with HIPAA regulations potentially stifling data-sharing progress.

Solution in Practice: MyCare / Impact Advisors / XSELL Case Study

MyCare (a CPF portfolio company that is an MA full-risk VBC provider with over 35K patients) partnered with Impact Advisors and XSELL to develop an Agentic AI workflow and Generative AI solution designed to alleviate these burdens. This AI-driven platform integrates comprehensive health records into standard EHR workflows, enables a digital health wallet for real-time ADT notifications and enhance Blue Button access to collectively address multifaceted challenges healthcare providers face. It progresses through five interconnected stages, each fully interoperable through a single AI deployment platform, enhancing operational efficiency.

- **In Care Coordination,** AI agents seamlessly manage communication and task distribution, reducing manual effort through full integration with the EHR. The AI agent reaches out to the patient or caregiver after a hospital admission to schedule a Transition of Care appointment and to address medication adherence needs.
- **Personalized Care Planning** leverages Generative AI to generate tailored plans with real-time updates, summarizing medical histories, from clinical and claims data, to save clinicians significant time and minimize hospital readmissions.
- **Patient Engagement** employs conversational AI for FAQs, reminders, and education, while fostering self-management.
- **Proactive Monitoring and Follow-Up** uses AI analytics to identify rising risk patients and automate follow-ups.
- **Addressing SDoH,** the AI agent assesses social risks (e.g., food insecurity, transportation needs) and coordinates community resources, automating interventions to lighten provider loads.

Value Demonstrated

- **Improved Care Team Responsiveness:** Reduced time between clinical handoffs and transitions enhances patient and clinician satisfaction.
- **Enhanced Patient Outcomes:** Increased adherence rates and clinical effectiveness, with better control of chronic conditions (e.g., HbA1c, BP).
- **Financial Benefits:** Reduced thirty (30) day re-admissions from 16% to 12%. Reduced administrative costs by 8-12% and care coordination expenses by 25-35%.
- **Staffing Efficiency:** Reduced clinician time spent on care plan development by 20-30%, with a 15-25% efficiency gain in patient education efforts.
- **Proactive Health Management:** Lower total cost of care by 10-20% through reduction in ER and hospital utilization, driven by proactive agent-based outreach and targeted clinical interventions.
- **SDoH Impact:** Increased utilization of community support resources, reducing healthcare utilization costs by addressing non-medical barriers.

Recommendations for CMS

MyCare's Agentic and Generative AI approach offers a robust framework to transform provider workflows by automating routine tasks, enhancing care coordination, and addressing SDoH, all while delivering measurable efficiency gains and bending the medical cost curve. When paired with broader solutions like digital health wallets it can significantly lighten the load on providers. CMS's support through incentives and policy reforms can amplify these benefits and as such CMS should consider the following recommendations:

- **(PR-1, VB-1)** Incentivize the adoption of AI-driven platforms through reimbursement models, drawing inspiration from the meaningful use program's success in driving EHR adoption with a time-phased, multi-stage incentive aligned to adoption, implementation, upgrade and consistent use.
- **(PR-2, TD-1)** Update CHERT policies to mandate default integration of records into EHRs, ensuring seamless access without additional steps.
- **(PR-1, PC-6)** Simplify Blue Button data access by allowing providers to use treatment consent, easing the burden on seniors.
- **(PR-1, TD-1)** Support the development of centralized ADT alert collectors and require hospitals to publish standardized setup instructions, minimizing administrative complexity.
- **(TD-4, PC-14, PR-2)** Address adoption barriers by exploring a single, interoperable identity standard, such as extending the Medicare Beneficiary Identifier (MBI), to streamline data sharing and overcome participation hurdles, leveraging vendor interoperability expertise.

5. Supporting Value-Based Care Goals

Fragmented event feeds, duplicative consent processes, and closed EHR tables hold back accountable care organizations. A single interoperability spine—built on TEFCA connectivity, FHIR services, and reusable patient consent—can unlock real-time insight, lighten administrative load, and improve clinical performance.

Challenges in Supporting Value-Based Care Goals

- **Event and Quality Alerts:** Patient identification services can flag and pass ADT, pharmacy, and laboratory messages directly into existing provider workflows to ensure primary care providers are alerted when e.g., patients are admitted or discharged from the hospital.
- **Reusable Consent Service:** Patients can provide consent once and permissions travel with the patient across all sites, HIEs and state lines.
- **EHR API:** Integration, particularly with leading EHR platforms, is often complex and demands significant technical expertise, creating substantial barriers to Value Based Care interoperability and performance.
- **EHR Data Egress:** Certified EHR technology must clearly expose standard data and reporting tables to facilitate integration with care navigation and analytics tools. Further, it must do so in a manner and format that is accessible and easy to use. Rather than hide APIs behind a complex registration process, advanced technical requirements, and out of date and incomplete documentation, API and bulk data exports should be turn-key, and accessible / enabled via a user-friendly front end within the EHR itself with no additional cost or intervention by the EHR vendor required.
- **Provider Blue-Button Proxy:** Expand treatment consent to allow access to Blue Button data and bypass beneficiary portal enrolment, enriching longitudinal records.

Solutions in Practice: Achieving Better Value-Based Care through Applied Technology Solutions

Some examples of how our VBC portfolio companies and technology solutions have addressed these challenges are:

- WellBe leveraged LLMs on patient contact data and patient targeting analytics with a specific focus on improving conversion rate (25% -> 30%), completion rate (60% -> 72%) and targeting top 20% highest risk / cost patients to engage to assess early disease burden and deliver high quality care.
- MyCare faced with the need to identify the highest risk patients in their population and allocate limited care management resources, developed a custom patient stratification algorithm. The data required by the algorithm includes real time data from their EHR, up to the minute event data from state HIEs and HIE aggregators, and historic claims data. To enable this functionality, the team had to internally solve for cross-source patient identification and matching, regional data source limitations (HIEs excluding specific facilities / hospital networks), and EHR data extraction and processing.
- Allymar delivers risk adjustment and quality improvement services to portfolio companies such as Atrio, MyCare, and P3 by combining technology, analytics, and AI (including generative and agentic AI). The platform identifies previously diagnosed and documented conditions that have not yet been submitted to CMS. It also analyzes a patient's full clinical profile (e.g. claims, medical charts, labs, radiology images and reports) to detect potential undiagnosed conditions and presents these as real-time, evidence-supported suggestions to the provider within the EHR at the point of care. On average, providers using Allymar's solution show a 30–40% increase in evaluation rates and an improvement in RAF accuracy of 20 – 30%. Compared to other solutions, Allymar stands out for its early detection capabilities in dementia, Alzheimer's, mental health disorders, and other rare diseases, as well as its ability to accurately assess the progression and staging of chronic conditions such as congestive heart failure and diabetes.

Recommendations for CMS

- **(VB-1)** Offer a modest shared-savings uplift to ACOs that submit timely high quality monthly data files.
- **(VB-2, VB-3)** Expand CEHRT to include additional tech capabilities to support overall goals of enabling VBC tools**.**
- **(VB-3)** Develop a nationwide standard for HIE participation to encourage interoperability and seamless data transmission across multiple exchanges.
- **(VB-8)** Require every hospital to join at least one QHIN and publish a single, public data access endpoint / API for reporting and data sharing purposes enabling ease of read / write access to VBC provider platforms to reduce burden on limited technical resources.
- **(VB-11)** Provide TEFCA onboarding vouchers for smaller ACOs to offset first-year connectivity costs and facilitate adoption among all healthcare providers.
- **(PC-9)** Establish a **Consent Clearinghouse** to advance Blue Button 3.0 to allow provider pull under HIPAA treatment consent, payment, and operations provisions.so that consent, and data can travel freely across state lines and remove redundant paperwork and maintain patient trust.

6. Real-World Evidence and Data Generation

Real-world evidence (RWE) is pivotal for advancing healthcare innovation, enabling better clinical decisions, and developing policies that improve patient outcomes while managing healthcare costs through insights derived from EHRs, administrative claims, patient-generated data, and other sources. CMS has long supported real world data generation though programs such as the Medicare Standard Analytic Files (SAF) and Chronic Conditions Warehouse (CCW), Qualified Entity (QE), and T-MSIS Analytic Files. CMS generated RWD has been broadly leveraged to support program integrity, quality of care evaluations, payment reform, and public safety initiatives for policy makers, payers, providers and other healthcare stakeholders.

The CMS Request for Information (RFI) seeks to understand how technology can support this process, particularly in expanding data access, improving data quality, and integrating data for value-based care (VBC) organizations. This response enhances the perspective on RWE generation by leveraging the capabilities of OMNY Health, a leader in providing comprehensive real-world data for researchers in health systems and life sciences organizations.

Challenges in RWE Generation

Several barriers hinder effective RWE generation

- **Data Quality and Completeness:** Ensuring RWE is reliable, and representative of diverse patient populations is challenging, especially when datasets lack standardization or completeness and RWD sources cover specific patient populations, impacting its utility for clinical and policy decisions.
- **Longitudinal Patient Linkage:** People move between payers, providers, and states; record-linkage breaks when identifiers change that erode multi-year follow-up that's critical for safety surveillance and comparative-effectiveness studies.
- **Coding Evaluation and Misclassification:** Mid-study shifts from ICD-9 to ICD-10, SNOMED updates, or local lab-code re-mapping can re-label exposures and outcomes overnight and interfere with real-world signal.

- **Tools for Analysis:** RWE in clinical data is voluminous and unstructured, especially clinical notes. These notes contain potentially valuable insights around disease progression, drug switching etc. but require advanced tools (such as LLMs trained on this data) to enable efficient analysis.

Real World Evidence Best Practices – OMNY Health Case Study

Centralized Data Access: OMNY Health provides a single, secure point of access to de-identified clinical data from over 85 million patients, reducing the effort required for feasibility assessment and data acquisition. OMNY has been expanding access to their data by partnering with other cloud and compute providers, such as Snowflake and Datavant / AWS. This growing infrastructure allows broad access to patient data, often linked to other sources of RWD, while providers, ASTP ONC, CMS and other stakeholders progress on data sharing at the point of care.

Improved Data Quality: OMNY Health ensures high-quality data by offering comprehensive, longitudinally rich datasets (seven years of patient history), which can be used for regulatory strategies, disease progression analysis, and sub-population evidence generation.

- **The platform supports a range of applications, including:**
  - Regulatory and access strategies with RWE from patients in all 50 states.
  - Disease progression and sub-population analysis for personalized medicine.
  - Post-market surveillance for manufacturers, tracking performance and ensuring compliance over seven (7) years of patient data.
  - An OMNY patent enables the secure and compliant access to clinical data by both pharma researchers and providers via “clean rooms” on core platforms.
- **Integration with Other Data Sources:** OMNY Health facilitates the alignment of disparate datasets, including claims, imaging and genomic data, enabling predictive modeling and supporting VBC data integration (VB-3). This is crucial for enhancing patient care coordination and outcomes.
- **Addressing Blue Button Challenges:** While OMNY Health does not directly replace Blue Button, its centralized approach complements existing data-sharing initiatives by providing an alternative, efficient way to generate RWE without relying solely on patient-driven tools, potentially easing the burden on seniors.

Recommendations for CMS

- **(TD-2, VB-3)** Encourage partnerships with data providers like OMNY Health to make comprehensive clinical datasets more widely available for research and innovation. By leveraging cloud-based platforms in FISMA compliant environments, CMS can facilitate secure, privacy-preserving data access that supports faster evidence generation, as seen in OMNY Health's approach.
- **(TD-9, VB-3)** Work with vendors to establish standards for data quality and completeness, ensuring RWE is reliable and representative. OMNY Health's model of providing high-quality, de-identified clinical data can serve as a benchmark for such standards, enhancing trust in RWE for clinical and policy decisions.
- **(VB-3, TD-2)** Promote the use of integrated data platforms to combine real-world data with other sources (e.g., genomic data) for value-based care initiatives. This can enhance patient care coordination, enable VBC organizations to track performance, and support outcomes-based care models.

- **(PC-7, TD-17)** Create one CMS-sponsored “evidence lab” where health-tech firms, providers, and payers can test new digital products on de-identified national data (e.g. AI innovation hub described above). Federal seed funding covers the secure environment; companies bring their algorithms and walk away with proof that meets coverage and regulatory standards.
- **(VB-4, VB-6)** Define a single, nationwide data package including claims, clinical records, social factors and patient-reported outcomes, and require every value-based contract to participate. Leverage the investment that CMS and other HHS agencies have made in developing RWD and the infrastructure that supports generating evidence from these sources. Pair the dataset with common measurement and reporting tools so all ACOs judge quality and cost the same way, speeding comparisons and scaling best practices.

7. Technology Ecosystem Guidance

Technology solution providers (e.g. EHR platforms) anchor healthcare’s data supply chain. Today’s landscape reflects a compliance with the “letter of the law” versus the “spirit of the law”, resulting in proprietary lock-ins, fragmented identifiers, and uneven standards, imposing hidden costs on providers, researchers, patients and caregivers.

Challenges in the Tech Ecosystem:

- **Closed EHR API:** Major EHR platforms still gatekeep bi-directional read / write access of providers’ own data, forcing costly middleware and leaving smaller systems unable to surface longitudinal data in near-real-time.
- **Patchwork Patient Identification:** Absent a universal identifier, providers juggle Medicare Beneficiary Identifiers (MBIs), state IDs, and vendor keys.
- **Misaligned Incentives:** EHR vendors profit from custom interfaces, not openness; without regulatory carrots or penalties, ROI favors proprietary modules over shared APIs researchers need.
- **Opaque Data Stewardship:** “Usage fees,” throttled export speeds, and unclear de-identification rules raise legal risk, slowing datasets destined for analytics pipelines and quality reporting.
- **Fragmented Standards Adoption:** Despite TEFCA, inconsistent ADT messaging and optional implementation guides leave edge cases unresolved; providers absorb integration rework instead of focusing on clinical insights.

Solutions in Practice: Governance / Trust / Security / Compliance / QA – Allymar Health Solutions

Robust governance is critical to ensuring that healthcare technology solutions are secure, compliant, and trustworthy. The "AI Safe Standard of Care™" framework from Allymar Health Solutions, provides a valuable model coupled with Impact Advisors responsible AI playbook. Vendors can support governance by:

- **Developing AI Responsibly:** Establish the foundation and develop standards for responsible AI development based on continuous reliability checks, safety simulations, fairness audits, penetration testing, interoperability reviews, cultural competence assessments and robust stress tests sustain high performance and public confidence.

- **Build AI for Clinical Performance and Transparency**: Mandate adherence to responsible AI standards to develop products with real-world cross-validation, clinician-run safety trials, and continuous live-dashboard monitoring guarantee the technology stays accurate, stable, and interpretable; Maintain a human-in-the-loop and improve explainability for clinician case reviews to accelerate adoption by skeptical end-users.
- **Secure, Fair and Resilient Operations for Trustworthiness**: Maintain access controls, perform adversarial stress-tests, bias audits across demographics, and HIPAA-aligned data-governance guardrails harden the technology and tools against misuse, ensure cultural competence, and lock in end-to-end regulatory compliance as it scales.
- **Providing Oversight Tools:** Vendors can develop platforms that enable organizations to implement roles outlined in the AI Safe Standard of Care™. These roles ensure responsible AI use, audit readiness, and alignment with legal and ethical standards.
- **Enhancing Compliance and Security:** Solutions that simplify compliance with regulations like HIPAA or address specific pain points such as streamlining Blue Button access for seniors via treatment consent, help organizations meet regulatory requirements efficiently.
- **Building Trust through Guidance:** To raise the bar on trust—especially as AI solutions move closer to clinical care and decision support—vendors should not only offer robust training, clear protocols, and comprehensive support frameworks (such as the human oversight and compliance frameworks) but also drive innovation through the establishment of a dedicated, measurable trustworthy AI certification framework. This trustworthy AI certification, potentially developed and facilitated in collaboration with Impact Advisors and Allymar as part of an overarching innovation program, would standardize accountability, transparency, and quality assurance, significantly bolstering healthcare organizations' confidence in adopting AI-driven clinical decision support solutions.

Recommendations for CMS

- **(TD-1, TD-2, TD-4)** Make full-write FHIR endpoints and TEFCA participation mandatory for vendor certification and bonus pools. Ending multi-HIE contracting slashes interface costs, delivers longitudinal data within days and can widen research cohorts.
- **(PC-14, VB-1)** Extend the Medicare Beneficiary Identifier to commercial plans and Medicaid. Pair the rule with “Meaningful Use–style” incentives so providers that prove clean match rates receive higher quality-measure payouts.
- **(PR-2, TD-6, TD-1)** Fund vendor-built, off-the-shelf tools—central ADT alert collectors, patient wallets granting one-tap consent—that small practices can deploy in a weekend. Tie grant dollars and shared-savings bonuses to active production use.
- **(TD-7, PR-3, VB-8)** Condition Medicare Advantage and ACO contracts on adoption of an “AI Safe Standard of Care”: board-level oversight, compliance-risk mitigation, dispute containment, and incident readiness. Require role-based dashboards (e.g., AI Safety Director) to document audits and model drift.
- **(PR-1, PC-6, TD-7)** Replace account-creation hurdles with treatment-based consent, uniform UX guidelines, and auto-renewing tokens. Provide vendor training grants so safety-net clinics master privacy, audit prep, and rapid incident response.